



Urology Record – Microscopic Hematuria Workup and Surveillance Plan

Service: Urology – Hematuria Evaluation Program
Facility: NorthStar Urology Associates (Network Partner)
Document Type: Longitudinal Hematuria Workup Summary
Coverage Period: 2021-08 to 2025-03
Most Recent Review Date: 2025-03-24

****Reason for Evaluation:**** Recurrent episodes of microscopic, painless hematuria identified on urinalysis. Patient is a never■smoker per chart and denies gross hematuria.

****History:**** Hematuria has been intermittently present over several years. The patient reports no dysuria, frequency, flank pain, renal colic, or constitutional symptoms. No occupational exposures to aromatic amines are documented. Prior evaluation included imaging and endoscopic assessment.

****Investigations (chronological summary):**** (1) 2021■08 urinalysis with microscopic hematuria; urine culture negative; urine cytology negative for high■grade urothelial carcinoma. (2) 2021■10 CT abdomen/pelvis hematuria protocol: no mass or stone identified in available summary. (3) 2021■11 cystoscopy by Urology: normal endoscopic evaluation; cytology pending/ultimately reassuring per longitudinal notes; recommendation for annual renal ultrasound surveillance. (4) 2022■12 renal and bladder ultrasound: normal. (5) 2023■12 renal ultrasound and urinalysis: normal; no hematuria documented that date. (6) 2025■01 urinalysis again shows microscopic hematuria; ACR within normal limits. (7) 2025■03■24 urinalysis WNL; urine culture negative; urine cytology negative; renal ultrasound normal; prostate volume reported 13.2 cc with post■void residual 63 cc.

****Assessment:**** Microscopic hematuria with reassuring urologic evaluation to date. No imaging evidence of renal mass; cystoscopy negative; cytology negative. Etiology remains undetermined.

****Plan:**** Continue annual surveillance renal/bladder ultrasound (next due ~2026■03) and repeat urinalysis as clinically indicated. Re-refer to Urology for any gross hematuria, new irritative symptoms, or risk■factor change.



Addendum / Comparison and Administrative Notes

Comparison statements reflect the most recent prior studies available in the network at the time of reporting. If external imaging exists, formal comparison may be limited until images are imported. Clinicians are encouraged to interpret findings in the context of the patient's broader longitudinal record and to correlate with physical examination and symptoms.

Quality Assurance: This report was auto-formatted from a structured radiology/clinic template and reviewed for internal consistency. Minor variations in measurements may occur due to technique, probe position, and interpreter variability.